

Chapter 13

Using Counseling Theories in Groups

The importance of understanding theories of human behavior cannot be overemphasized when leading any group that focuses on support, personal growth, life decision-making, counseling, or therapy. Groups on anger, stress, anxiety, and depression will be effective only if the leader can use theory to help members understand their thoughts, feelings, and behaviors. To a lesser extent, it is helpful to have some understanding of one or more theories when leading psychoeducational groups and certain discussion groups.

No leader should lead counseling groups without an in-depth knowledge of counseling theories. Unfortunately, many people who lead therapy groups have neither experience in individual counseling nor a background in counseling theories. These group leaders have some information about group process and group dynamics and they rely on the members to be the agents of change. These groups are usually minimally helpful and sometimes even harmful. Leaders of counseling and therapy groups need to know how to help members with their pain. For instance, in groups for sexual abuse victims, agoraphobics, suicidal patients, sex offenders, drug abusers, and children of alcoholics, members need a leader who has a theoretical understanding of their pain because the members have no idea how to feel better. We encourage leaders not only to use theories during the sessions, but also to teach group members the basic tenets of one of the major counseling theories. We usually teach rational emotive behavior therapy (REBT) and transactional analysis (TA) because these theories are very valuable in understanding human behavior and can be learned rather quickly by members regardless of their age.

Very often in therapy groups, the focus is held on one member for 10 to 20 minutes (sometimes longer). During the time when the group is focusing on

a member, the leader has two basic choices: to have the members try to help the working member by using one or more theories that have been taught, or to conduct a few minutes of individual counseling while engaging the other members in some way. To conduct individual counseling, the leader needs to have the theoretical knowledge and skills to take the session deep enough so that the members are very involved even though they may be simply watching and offering comments elicited by the leader. For leaders who do not yet have the ability to conduct interesting and engaging counseling, we suggest using the members' personal experiences and expertise and the leader's knowledge of theories. Rarely is it a good idea to conduct slow-moving individual counseling while members watch and listen, although unfortunately, this is done quite often and the members become bored.

The purpose of this chapter is to briefly summarize several contemporary theories that we believe have particular relevance for group work and then focus on examples of how each of these theories may be applied in group situations. It is our assumption that most readers will have already studied counseling theories and only need a review. We also realize that there is much more that a leader can do with each theory, but we mainly wanted to give you some initial ideas about how to use each theory in groups. As you use the theories, you will develop many of your own ways to implement them. We also realize that there are many other theories that are useful in groups that we do not cover in this text so you may want to seek out specific books and articles on how the different theories are used in group counseling. As we have said numerous times, we strongly believe that *knowing counseling theory is essential for leading counseling and therapy groups.*

Cognitive Behavioral Therapy

Cognitive behavioral therapy (CBT) is based on the principle that one's thoughts cause one's feelings. There are several approaches to cognitive behavioral therapy, including rational emotive behavior therapy, rational behavior therapy, rational living therapy, cognitive therapy, and dialectic behavior therapy. Two of the biggest names associated with CBT are Ellis (REBT) and Beck (cognitive therapy). In this chapter we focus on REBT. Other cognitive therapies would be conducted in similar fashion in that the principles, terms, and techniques would be taught and the members would be asked to look at how their thoughts cause their feelings.

Rational Emotive Behavior Therapy

One of the leading theories in the field of counseling is REBT, founded by Albert Ellis. It is an excellent theory for use in groups because it can easily be taught to the members, who can quickly learn how to apply it to

themselves and to other members. The theory is based on the premise that *thoughts cause feelings,* and in groups, the leader helps members to focus on changing their feelings by looking at what they are telling themselves. Thoughts such as "I should always be the best, and if I'm not, I'm a failure" or "I am a failure because I'm divorced and I will never be happy again" are examples of the kinds of things that might come up in different groups. REBT is quite helpful for children, adolescents, and adults because it helps them to feel more in control by knowing that their feelings are caused by their thoughts.

The Model

Ellis developed an ABC model of understanding feelings and behaviors. The letter A represents the *activating event* to be evaluated. The letter C represents the *consequences* or the emotional reaction experienced. B represents the *beliefs* used in evaluating the activating event (A) and ties A and C together. It is not the occurrence of an event that causes feelings, but rather the person's interpretation of that event. Most people have never examined the negative, exaggerated, self-defeating self-talk in which they engage. Most people believe that A causes C, but in fact it is B: What they are telling themselves is what causes C. Once the members learn the theory, they often are the ones to help other members challenge, dispute, and replace the negative self-talk. For example, if Horace is talking about feeling very shy at a dance and not asking anyone to dance, using the model, the leader could lay out the problem on a whiteboard like this:

(event) A = being at a dance
(self-talk) B = I'm ugly; no one would ever want to dance with me. It would be awful if someone turned me down if I asked her to dance!
(feeling) C = shy; fearful, bad (behavior = stand in corner)

Teaching REBT to the Members

In group counseling where the leader plans to use REBT, the first thing that needs to be done is to teach the theory to the members. We suggest two ways to introduce and teach REBT to the members. The first is to take a problem that a member has brought up in the session, such as the one with Horace, and write it out and teach all the members the ABC model and the theory behind the model. In other words, use a real problem from one of the members but take considerable time to teach the idea that thoughts cause feelings.

A second way to get members to focus on this theory is to start by asking them, "Where do feelings come from?" Many will say from "others," "the heart," "the past," or "situations." This leads to presenting the idea that *thoughts cause feelings.* We suggest using a whiteboard and handouts that lay out the ABC

model. Then discuss some examples of how the model works with such feelings as anger, guilt, or worry. For example, members begin to see how thoughts cause feelings by showing them how it's not traffic that causes anger but what they tell themselves about traffic that causes them to feel angry. The key is to get members to start seeing that their thoughts cause their feelings or to at least consider the notion that it is not their love partner or boss or parent who makes them mad, but rather they make themselves mad by what they tell themselves.

To introduce REBT and to show group members that it is not the event that causes feelings but rather the self-talk, one example that can be used is to divide the group in half and tell one half to pretend that they are excellent students in school—make all As—and the other half to pretend they are D students; then have them assume that they all make a C+ on a test. Then ask them how they feel. The A students will say "bad" and the D students will say "good." The leader can then ask the members, "If events cause feelings and you all experienced the same event (got a C+), shouldn't you all feel the same?" This often generates discussion about the effects of our thoughts on our feelings, which is the basis of REBT. Also the leader can comment on how people often tell themselves all kinds of "shoulds," "oughts," and "musts" and also engage in exaggerated self-talk that creates feelings that lead to negative or destructive behaviors.



GROUP COUNSELING SKILLS: Theories—REBT

Go to video 13.1 and watch an introduction to the use of theories in groups and a segment where the leader uses REBT.

By using the whiteboard and other creative techniques, such as two chairs (one marked *rational self-talk* and one *irrational self-talk*), a leader can help members visualize the basic principle of REBT that people have a choice as to what they tell themselves. Also by using a handout with some basic REBT information on it, members usually can understand the theory. To help them further grasp the notion, we sometimes use Ellis's list of irrational ideas that people tell themselves. We ask members to read the list and circle the irrational ideas they currently tell themselves. This leads to further discussion about the importance of our self-talk when it comes to understanding our feelings and behavior. Granted, many, at first, don't buy the idea that their thoughts cause their feelings, because most people believe that people and events upset them. They have never really considered that they may be causing and sustaining the majority of their negative emotions. Here is the list of Ellis's main irrational sentences. We also provide a list for kids based on the original list of 15 irrational ideas.

Ellis's 15 Irrational Ideas

1. It is a dire necessity for an adult human being to be loved or approved by virtually every other person in one's life.
2. One should be thoroughly competent, adequate, and achieving in all possible respects if one is to consider oneself worthwhile.
3. Certain people are bad, wicked, and villainous and they should be severely blamed or punished for their villainy.
4. It is awful and catastrophic when things are not the way one would very much like them to be.
5. Human unhappiness is externally caused and people have little or no ability to control their sorrows and disturbances.
6. If something is or may be dangerous or fearsome, one should be terribly concerned about it and should keep dwelling on the possibility of its occurring.
7. It is easier to avoid than face certain life difficulties and self-responsibilities.
8. One should be dependent on others and needs someone stronger than oneself on whom to rely.
9. One's past history is an all-important determiner of one's present behavior, and because something once strongly affected one's life, it should indefinitely have an effect.
10. There is invariably a right, precise, and perfect solution to human problems and it is catastrophic if this perfect solution is not found.
11. One should become quite upset over other people's problems and disturbances.
12. The world should be fair and just and if it is not, it is awful and I can't stand it.
13. One should be comfortable and without pain at all times.
14. One may be going crazy because one is experiencing some anxious feelings.
15. One can achieve maximum human happiness by inertia and inaction or by passively and uncommittedly enjoying oneself.

Common Irrational Beliefs of Kids

1. I must be liked by everyone, and if I am not it is awful and I can't stand it.
2. If someone calls me names, it must be true and I can't stand it.
3. I should be the best at everything I do and if I am not, I am worthless.

4. Some people are bad and I have to dwell on how to get back at them.
5. It is awful when things are not the way I would very much like them to be.
6. My unhappiness is caused by others and I have no ability to control my unhappiness and have no ability to make myself happy.
7. It is easier for me to avoid certain troubling situations than to face them.
8. I cannot depend on myself—I have to depend on others for my strength.
9. My past causes me to be the way I am and there is nothing I can do about it.
10. There is a perfect solution to every problem and it is terrible if I cannot figure out the perfect solution.
11. I must become upset and stay upset over other people's problems.
12. Things should be fair and if they are not, it is awful and I can't stand it.
13. I should never be uncomfortable or inconvenienced and when I am it is awful and I can't stand it.
14. I can achieve and be successful even if I do nothing and have no plan of action.
15. It is my fault if my parents fight (drink, are getting divorced).
16. Because I am adopted (in foster care, have less money), I am less than other kids.
17. Because he/she did that to me (physical, sexual, emotional abuse), there is something wrong with me and I don't deserve to be happy.
18. If I love my stepdad (stepmom), it means I don't love my dad (mom).

These lists can be used to generate discussion about members' irrational beliefs. The discussion of the different sentences can lead to personal work by one or more of the members if the purpose of the group is problem solving, counseling, or therapy.

Using REBT in Groups

Clarifying the event, person, or situation (A) Whenever an issue is brought up, it is usually helpful to clarify the specifics of the situation that are troubling the member. For example, if a member, Anna, is talking about how much her in-laws upset her, the leader may ask Anna if it is mostly the mom, the dad, or both, and how often she is upset. The leader may have the members ask questions to help Anna clarify what she means. The leader could say to the group, "Using the ABC model, what do you hear as the activating event?" By using this process, it involves the members and it keeps them thinking about how this theory can be used with most situations. As a result of this focusing process, Anna may say something like, "It's really just my mother-in-law and it's mostly about my parenting of my daughter."

Clarifying the feelings and/or behavior (C) Once it is clear what the situation is, usually the next step is to clarify the feelings or behaviors associated with the event. The leader may ask questions or the members may ask questions to help the working member specify her feelings. In the example of Anna, after hearing her describe her feelings, the leader might say, "Anna, what would you say is the main feeling?" She might say, "I feel hurt" or "I feel very angry" or both. The leader could say to the group, "What do you hear as the main feeling that we should help Anna with? What is the C in the ABC model that you think Anna may need to work on first?"

Clarifying the negative self-talk (B) Once A and C are clarified, the leader concentrates on identifying the self-defeating, not true, irrational self-talk that the member is telling herself. In this example, the leader would focus the group on helping Anna figure out her self-talk, especially if Anna can't seem to figure it out herself. The leader could say something like, "What do you think Anna is telling herself that is causing her to feel the way she does?" Members may offer suggestions such as, "I must have my mother-in-law's approval and if I don't it means that I'm a bad parent," or "She should not treat me the way she does. She has no right to criticize me."

Changing the feelings by changing the self-talk The most important part of REBT is to get members to see that what they are telling themselves is not true. In our example, the leader would facilitate work with Anna by trying to get her to tell herself something different—something that is rational and true. The leader would seek input from the other members about what it would mean if her mother-in-law did not approve and what alternatives Anna could tell herself about her mother-in-law's behavior that would result in her not feeling so angry. The benefit of using REBT in groups is that members can get involved in helping the working member because they would be familiar with the model that the leader is using. We believe one of the most important advantages of using REBT is that members can use it to help other members and also the members may apply it to themselves while trying to help another member.

EXAMPLE 1

The school counselor is leading a personal exploration group of sixth graders. The group members know REBT. The group is focused on Joey, who has just learned he has been cut from the school's soccer team.

LEADER: *(Standing at the whiteboard and writing)* So you got cut from the team. We'll write that here, at A. And how are you feeling?

JOEY: Sad. And mad. It wasn't fair that I got cut. There were two guys that made it and they were worse than me. The coach doesn't like me.

LEADER: *(Writes "sad" and "mad" at C. Then the leader looks around the group, ready to write at B.)* Does it make sense that Joey feels both mad and sad?

Bruce: Yeah. But life isn't always fair; at least that's what we talked about last week from that list.

Leader: *(The leader writes under B: "It isn't fair that I got cut")* What do we need to help Joey with?

Damita: We have to help him dispute his self-talk.

Leader: Okay, let's stay at B. What else is Joey thinking or telling himself? Joey, I'm going to get the group's ideas and then get you to clarify if what we write up here is what you are telling yourself.

Josh: Because he got cut, he's not athletic. And that would be terrible.

Leader: Good. Let's do one or two more.

Paco: He could be telling himself that because he got cut he'll never be successful at anything. Also, he said he thinks the coach doesn't like him.

Leader: *(Writing under B)* Joey, look at our list at B. Is this sort of what you're thinking?

Joey: Yeah. All of that.

Leader: Are you willing to work with us to see if these are true? And if they are not true, will you work to stop telling yourself these thoughts?

Joey: Yeah.

Leader: As you look at the list, which one is the toughest to tackle?

Joey: I think that Mr. Franks doesn't like me. I know I'm not that good at soccer. But I like Mr. Franks a lot. I don't want him to think I'm a jerk.

Leader: *(Draws a line under "The coach does not like me." Also adds, "He thinks I'm a jerk!" Looks around the group.)* Anyone want to start disputing this?

Paco: Because the coach cuts you from the team, it just means he can carry only a certain number of players, not that he doesn't like you.

Leader: *(Using a Not True/True grid, he writes Paco's statement next to "The coach does not like me. He thinks I'm a jerk.")*

Not True	*True*
It isn't fair that I got cut. *The coach does not like me.* *He thinks I'm a jerk!*	*He could only pick so many players.* *It has nothing to do with him liking me.* *His not picking me does not mean he thinks I'm a jerk—just not one of the best soccer players.*
Mad, Very Bad	*Better*

In all kinds of groups, the same general format as this one can be used for members to help each other. The leader focuses on getting members to help other members dispute irrational sentences that are causing problems in their lives.

EXAMPLE 2

The group is made up of 10 members in an inpatient facility where the membership changes on a daily basis due to new admissions. This group is one of three groups that the members will be in that day. It is important that the leader have some activity that can be of benefit to most members. Using the list of irrational sentences given previously can usually create a productive group.

LEADER: I'd like to start by having you look at this list of sentences and pick out any that you think are true or pick out ones that you want to discuss for one reason or another. If you have trouble reading them, I'll help, or maybe your neighbor can help you. *(Waits a minute or so until most appear to have finished the list.)* Anyone have one?

KATE: I do. Number 9—One's past history is an all-important determiner of one's present behavior and because something once strongly affected one's life, it should indefinitely have an effect. I think what happened to me when I was young will always cause me to hate men.

LEADER: Did anyone else pick that one?

DICK: I did. My mom's murder causes me to be depressed every day.

JUNE: I picked that one. I would not be here if it weren't for my husband. He ran off with my cousin and now they are living in Canada. I'll never get over that.

LEADER: I think we'll work with this one sentence today and then focus on others tomorrow when we meet. Many of you believe that your past has determined your current life and I'm going to present a whole other way to look at your past. The truth is that it is not your past or certain events that make you feel bad but rather it is what you are currently telling yourself about the events that are causing you to feel bad.

KATE: Like what? If those men would not have abused me, I would not think all men are scum.

LEADER: Let's look at that, and I want all of you to help out with this. *(To the group)* Do you think that Kate is right about all men are scum or that those men were scum?

MARIO: I'd say it was those men. I'm a man and I never treat women badly. In fact, they always say I'm too nice.

LEADER: Mario, maybe later we can focus on your being too nice but for now, I think you made a good point. I say this to you, Kate, and really to all of you. The key to healthy living is to challenge and dispute self-talk in your head that makes you feel bad. Many of you have been telling yourselves things for years and I hope today's group gets you started in disputing the self-talk in your head that is not true. Kate, do you want us to help you with this?

KATE: I don't like feeling the way I do.

LEADER: *(To the group)* What do you think Kate is telling herself about her past and what can she tell herself?

The leader would focus on Kate and other members' irrational, self-defeating self-talk about their past and try to get the other members to dispute the thoughts of a member who brings up different ideas. The leader would try to get the members, such as Kate, June, and Dick, to realize that what happened to them was very unfortunate, but they are not less of a person and they can have an enjoyable and fulfilling life.



GROUP COUNSELING SKILLS: REBT and Test Anxiety

Watch video 17.2—using REBT in a psychoeducational group pertaining to test anxiety. This shows teaching high school students about test anxiety using a variety of creative techniques and REBT theory. In this extended session, the leader shows the beginning, middle, and closing phases of a session.

Suggestions for More Reading

There obviously is much more to REBT than what we covered in these few pages. Our goal was to briefly review the theory and show how it can be used in groups. For more information about REBT, we suggest reading *A Practitioner's Guide to Rational-Emotive Therapy* (DiGiuseppe, Doyle, Dryden, & Backx, 2013). An excellent book for school counselors is *Rational Counseling with School-Aged Populations: A Practical Guide*, by Wilde (1992).

Reality Therapy

Reality therapy, based on the work of William Glasser, proposes that human beings are responsible for making choices about behavior and that such choices are intended to meet five basic human needs: survival, love and belonging, power (achievement), freedom (to make life decisions), and fun (Wubbolding, 2000). Glasser's original work in reality therapy has evolved into what he terms

choice theory, with the focus on the importance of interpersonal relationships in the satisfaction of human needs (Glasser, 2000). Of particular interest to the group leader is Glasser's belief that many "symptoms," such as nonclinical depression and anger, are behaviors intended to control others and the environment. Therefore, his theory considers conduct such as "angering" and "depressing" as active choices, which can be altered in favor of more constructive behaviors.

Choice theory focuses little on the past and relies on the group leader to play an active role in helping members to make better choices and to develop more positive plans of action. When using reality therapy, the leader's ultimate goal is for members to accept responsibility for their behavior and to make better choices. In order to facilitate change, the member must understand her inseparable, but distinct, components of behavior: *acting, feeling,* and *thinking,* along with basic *physiology*. Reality therapy contends that *physiology* and *feelings* are more difficult to modify than *acting*. Reality therapists believe that most of the time we can choose how to act, regardless of how we feel emotionally. To a lesser extent, we can also choose what we *think*.

Using Reality Therapy in Groups

Wubbolding (2000) identified an acronym to assist the reality therapy group leader in helping members work toward changing their behaviors in a more positive direction. The acronym WDEP stands for *want, doing, evaluation,* and *plan.* The first few times the leader uses this in the group she may want to outline it on the whiteboard, putting up the letters and filling them in as the member works through the process.

Identifying *wants (W)* is an important starting place. When a member brings up an issue or problem with which she wishes help, the leader who is using reality therapy will ask in various ways: "What do you want?" If the response is general or unclear, the leader will press forward by asking, "What do you *really* want?" In a group setting, in particular when members have bonded with one another, the leader may have one or more members ask the working member questions to clarify what she really wants.

Once a member becomes clearer regarding what she wants and that "want" is something that the member has control to change or achieve, the leader and members ask questions to get the working member to state specifically what she is currently *doing (D)*. The next step in the WDEP strategy is *self-evaluation (E)*. This stage requires that the member examine the effects of specific behaviors that she is currently doing. The leader or members ask "Is what you are doing getting you what you want?"

The final and most important stage is *planning (P)*. Here, again with the help of the group, the member sets up both short- and long-term goals that incorporate her new behaviors. It is essential that plans be simple and specific in detail, realistic, involve action (as opposed to avoidance behavior), not be dependent on another person or event to be carried out, and have a start time along with criteria to determine success. Group members help with the development and

refinement of a plan and assessing its success. The presumption is that no plan is magic, that a member who is committed to change will be willing to accept partial success and continue to revise her plan to move closer to the successful fulfillment of her needs.

As we have mentioned, behavior change does not occur through magic but requires a continuous process of refining needs, taking action, evaluating, planning, and reevaluating outcomes. As members establish their goals and formulate plans, the group setting provides an excellent venue in which members receive feedback from one another and the leader in order to better meet their needs. The atmosphere is noncritical, is non-blaming, and focuses on what changes will help meet a need or goal rather than on why success was not achieved. Effectively applied, reality therapy both identifies and tracks a member's behavior to enhance the chances of success. Reality therapy also stresses the importance of accepting responsibility for the consequences of one's behavior.



GROUP COUNSELING SKILLS: Theories—Reality Therapy

Go to video 13.3 and watch a segment where the leader uses reality therapy and WDEP.

■ EXAMPLE 1

In a rehabilitation counseling group, Joe verbalizes a desire to return to work. He points out, however, that he won't be allowed to return to work until he completes 3 weeks of physical therapy, which he says he is unable to attend because he lacks transportation. After several members make suggestions regarding transportation that Joe dismisses as unrealistic, the leader decides to help Joe clarify what he *really* wants.

LEADER: Joe, everyone in our group talks about the same goal of returning to work. It's like, that's what we're supposed to say. *(The leader uses a soft, slow voice, expressing care in his tone.)* If we were to ask you what you *really* want, deep down inside, what would you tell us?

JOE: *(Tearing up)* I would like to spend more time with my kids. *(Pause)* And I don't want to get hurt again on the job.

LEADER: *(Recognizing that Joe is actually meeting his needs by staying home, the leader wishes to help him determine whether his choices are realistic. Again, using a kind, supportive, and nonjudgmental tone)* Can you afford to stay at home just now?

JOE: *(Shakes his head no)* Agnes is working two jobs. It's not fair to her. She's not getting any time with the kids at all.

LEADER: Let me ask you this question again. What do you really want?

JOE: I'd like to work at a job where I'm not so tired when I get home and where there is less chance of getting injured.

LEADER: Is avoiding the physical therapy helping you in that direction?

JOE: *(Shaking his head)* No.

LEADER: As you become more physically fit, we can talk about options for work. *(Turning to the other members)* Joe had the courage to say what he really wants just now. How are others of you feeling about where you are in your rehabilitation process?

TOM: I realize that I'm not being honest with what I want.

COOPER: I know what I want. I realize that what I'm doing is never going to get me what I want, which is a better job.

LEADER: When we finish with Joe, we'll work on both of those things.

■ ■ ■

■ EXAMPLE 2

In this example, we see a group of five women who have been mandated by the court to attend a counseling and parent education group for eight sessions to work on improving their parenting skills. This is the third session. The leader has explained the WDEP model. One member, Roxie, has said she wishes she could be more patient.

LEADER: Roxie, we've taken care of the "W," your wish to be more patient. And we know that a lot of what you've been doing with your kids is yelling, being short with them, and being very verbally abusive. And you've said you know this will not get you what you want. Can we help you develop a plan to get your patience under control?

ROXIE: That would be great.

DARLA: Has there ever been a time when you were patient with anyone?

ROXIE: *(Thinking)* I guess when my grandmother was sick in the hospital, I would go there when I could. She couldn't feed herself and I sat with her.

LEADER: You sat with your grandmother. What else were you doing?

ROXIE: Putting small pieces of food in her mouth, waiting for her to chew.

DONNA: When you were waiting, were you thinking, "Come on Grandma, chew it up so I can get out of here?"

ROXIE: *(Laughing)* No, no. I was very calm. I was thinking, this is my grandmother, I love her, and she needs me now.

LEADER: *(Looking around the room)* Is this a good start for Roxie?

SUSAN: Yeah. She knows what being patient is, how it feels.

YOLANDA: She's in touch with her love part. She knows she can show love and be patient.

LEADER: Right. Two behaviors you were doing, Roxie, might be helpful with your kids. You were sitting down. And you were waiting for your grandmother rather than trying to make her do something. How might those two behaviors be helpful?

ROXIE: I don't take time to sit with my kids. And, you're right, I always want them to do something instead of listening to them when they want to tell me something.

LEADER: If you were doing those two things, sitting and listening, would you be showing more patience?

ROXIE: Yes.

LEADER: Now let's make a plan. Let's think about each of your children. What specifically will you do? And when will you begin? *(To the members)* While we try to help Roxie with a specific plan, I want each of you to think about some specific things you want to do differently with your kids.

YOLANDA: I can think of some things I need to do.

LEADER: Good, let's first work with Roxie then we'll work with you.

■ ■ ■

■ EXAMPLE 3

This is the fifth session of a group of four teenage boys in a detention center.

LEADER: Today we're going to talk about basic needs that all of us have. This famous doctor came up with these and I think we can have a good discussion. I want you to look at this list of basic needs and I want you to think if these needs are currently being met here at the center and at the place you were living before being sent here. Here's the list.

Freedom and Independence
Love and Belonging
Sense of Power and Achievement
Fun
Survival

HECTOR: I'm not sure what all these mean.

LEADER: That's fine. Let's take one and I'll tell you what it means and then we can talk about it. Let's start with love and belonging. Everyone needs to feel this and when you don't, you often will fight, seek out a gang, or become a bully. Does any of this sound familiar?

HECTOR: I've never felt love from anyone. My mom was busy doing men all the time so she never had time for me. I'm used to not being loved.

GEORGE: I'm like Hector. I've already been in three foster homes. My last foster parents tried to love me, but they were too into religion for me. That's not me.

ALEX: I feel like people here care about me more than I've ever had. All this group stuff gets us close. I've said things in group that I've never said before, and when I found out my brother was killed, it felt good that people just sat with me. I've never had anyone do that.

JOHN: I joined the gang because my family didn't care about me. I don't know if I should go back to it when I get out.

LEADER: These are all good points. I think we'll spend this session just talking about the love and belonging need and then discuss the others tomorrow or the next day. What is the best way to find love and belonging? Also, who in your life can provide this for you in a positive way? John, I agree that it probably is not a good idea to go back to the gang for a sense of belonging. I'm hoping you can develop a better plan. What are your thoughts on all this?

The leader then holds the focus on this topic and uses the WDEP model to help each of the members.

Suggestions for More Reading

Glasser, W. (2000). *Counseling with choice theory: The new reality therapy.* New York: Harper Collins.

Wubbolding, R. E. (2000). *Reality therapy for the 21st century.* Philadelphia, PA: Brunner-Routledge (Taylor & Francis).

Adlerian Therapy

Alfred Adler, rather than believing human behavior to be controlled by instincts, as did Sigmund Freud and others of his time, suggested that much of who we are is the result of family and social interactions throughout early childhood. Adler is known for his belief that a person's birth order has a major influence on development of personality. Adlerian psychology, also referred to as *individual psychology*, emphasizes the notion that *all behavior is purposeful.* Behavior is intended to overcome a sense of perceived inferiority (feeling *less than*) resulting from the inability to be "perfect" or as good as others during childhood.

As children strive to compete with older siblings, friends, and adults in performing certain tasks, they often lack the developmental skills necessary

for success and develop a feeling of inferiority. Also during the development years (0–10 years), children attempt to find their place or role in the family. Many factors influence this process, including birth order, intelligence, attractiveness, athletic ability, parental and grandparental preferences, and gender. As a result of this striving and the level of a child's perceived success, the child will draw conclusions as to what is valued in the family. For example, a child may conclude that to be loved she must always please others, or accumulate great wealth and power, or play a traditional gender role, such as rearing children. A group leader using Adlerian theory will focus members on their "private logic," which includes mistaken goals and guiding fictions about themselves, the world, and others. Adlerians believe that if people can understand the purposefulness of their behavior, they have a much better chance of changing behaviors that are not useful or helpful.



GROUP COUNSELING SKILLS: Theories—Adlerian

Go to video 13.2 and watch a segment where the leader uses Adlerian Therapy.

Using Adlerian Therapy in Groups

We present a number of examples to further explain the theory and show how it may be used in groups.

■ EXAMPLE 1

This is the fourth session of a group of eight high school students who have an alcoholic parent. The leader decides to focus for a few minutes on some of the Adlerian concepts about birth order.

LEADER: *(After the warm-up period)* I thought today we would take a break from talking directly about your parents and the effects of their drinking, and we'd focus on you and your siblings. I do think you will find this interesting and helpful. We are going to talk about your birth order and how it affects your thinking and behavior in your family. Raise your hand if you are a firstborn child. *(Leader waits and looks around the group.)* What effect does it have on you? *(Leader gets a couple of comments.)* How about a second born? *(Checks hands and gets a couple of comments.)* Who was a middle child? *(Again looks around and sees members doing likewise. The leader continues through the youngest and the only child.)* There is a theory that believes that your birth order has a major affect on how you see things. First, I'm going to have you get in pairs or threes and talk about how you think your birth order is having an

impact on you and then we'll get back and discuss your thoughts and your reactions. It's important to realize that you act the way you do in part due to your birth order.

The leader has now generated interest, gotten the members involved in the process quickly and, perhaps most important, has members aware of similarities and differences among themselves regarding birth order. Next, the leader may ask members to share what it is like to be a firstborn, and so forth. Depending on the makeup of the group, certain characteristics will be described. As this exchange continues, the leader will elaborate on typical characteristics of the different birth order positions.

The firstborn child is often independent, identifies with parents, and may take on responsibility similar to adults, especially in caring for younger siblings. A second child may compete with an older sibling or go off and develop her own "place" in the family. For example, if the firstborn does well at academics, the second child may attempt to achieve at athletics or in some other area, depending on the opportunities available. Should a third child come along, the second child becomes the middle child and may feel "squeezed," competing with an older sibling and striving for the attention that is now going to the new arrival. Middle children sometimes develop the role of "negotiators," that is, arbitrating between the older and younger siblings or between siblings and parents. The youngest in a family is generally attended to by older siblings, parents, and grandparents and may remain the "baby" of the family. An only child may have characteristics similar to a first child: independence, identification with adults, responsibility, and high achievement.

There will be many exceptions to the general rules of birth order, and the role of the leader using Adlerian theory is to help the members consider the extent to which these characteristics work for or against them in their present lives. It is important for the leader to remember that birth order characteristics are never rigid and may vary greatly, depending on age gaps between siblings, children's physical health, demands on parents, divorce and remarriage, and so forth.

Dialogue following the discussion may look something like this.

LEADER: Any thoughts or comments about all this discussion?

RUDY: I never have thought about any of this. I always feel like the peacemaker when Dad drinks, but now I see that being the middle child may have something to do with this. Very interesting. I'm going to really think about this.

TOM: I've always feel the burden of being the good kid because my brother is getting in all that trouble. It never occurred to me that I might be the way I am because of the way he is. I feel Mom can't take much more so I have always tried to be good.

GLORIA: I always knew I resented my stepsister but now I have a better idea why—she took my place as the firstborn! I want to talk more about that.

LEADER: That's good, Gloria. The key for all of you is to realize that each of you behave in certain ways due to your birth order. What we want to do here is to examine your private logic, guiding fictions, and mistaken goals. It's obvious that many of you have things you want to talk about.

The leader would then focus on members' birth order and how their siblings influence who they are and how they think about themselves.

EXAMPLE 2

This is a group for women who suffer from social anxiety. It is midway through the third session.

LEADER: I think what would be helpful would be a discussion about how each of you found your place in your family and tie that in to your current fears and anxiety. This famous psychologist thought this was a very important thing to understand. *(Pause)* So how would you answer this question: When you were 6 or 8 years old, how would you say you found your place? For example, you might say "being the peace-maker," or "being good."

CAROL: There was so much fighting; I tried to be invisible and just blend into the woodwork. I was one of eight kids.

LEADER: We'll come back to that. Others, how did you find your place?

HILARY: Mom made sure that we were quiet all the time because we didn't want to ever wake Dad up. I was the quiet, good kid, but my brother was always messing up and getting in trouble with Mom or Dad.

CHAR: I always felt inferior to my sisters. I had an older sister who was a superstar and great looking, and a twin who was better at everything. When I went with them, they talked and I just listened and hoped someone would talk to me.

ASRA: In my culture, women were considered less than men, especially at social gatherings or at the mosque, so I learned to be quiet and stay out of the way.

LEADER: What are all of you thinking as you reflect on what's being said or what you said?

CHAR: That we each found our place in different ways because of our home situation or siblings.

LEADER: Exactly. I think it would be useful to help each of you examine the development of your private logic regarding how you found your place when you were young. As a kid, it may have made sense to be quiet and afraid, but now that you are grown and live in a different environment, it no longer makes sense. The purpose of this group is to

help you explore the purposefulness of your behavior so that you won't be afraid of social situations. Who wants to share their thoughts?

ASRA: I will. I'm always afraid. Everyone says I'm very shy.

LEADER: I want you to think for a moment what your life might be like if you chose not to find your place by being shy. In fact, I would like to ask you to try something. *(The leader pauses for emphasis.)* I would like you to visualize yourself as the opposite of shy. How would you be?

ASRA: An assertive, confident person.

LEADER: Could you act like that in here? What is something an assertive, confident Asra would do or say?

ASRA: *(Thinks, looks around the room and then looks directly at two of the members.)* You two are buddies and I know you usually go out after the group. I'd like to go with you sometime.

CONNIE: Sure. How about this evening?

LEADER: Let's talk about how this feels to you, Asra.

ASRA: It feels good. I wish I could do this out in the world. It is safe here.

LEADER: What you are saying applies to everyone here. Your shyness or fear made sense growing up, but what I want all of you to see is that it does not make sense now that you are grown women. You do not have to find your place now like you did when you were growing up.

CHAR: This makes so much sense. I just thought I was born this way. I never knew I could be any different.

The leader would focus on helping members see their private logic regarding people and social situations and try to help them see that circumstances are now very different.

■ ■ ■

■ EXAMPLE 3

This is the second session of a group of seventh graders focusing on improving self-esteem. It is 10 minutes into the session.

LEADER: I want to talk about an important thought process that influences self-esteem. It is called *guiding fictions*. I'm going to explain it and then have you think about any guiding fictions that you have. A guiding fiction is a false belief learned as a child that is currently directing your life. Guiding fictions may include beliefs such as, *I must be perfect to succeed or be loved. Because my parent never talks to me, there must be something wrong with me. There is no way I can ever amount to anything.* Most of you are unaware of some of your guiding fictions, and we are going to try

today to help you get in touch with some of them. Take a minute and try to think of any thoughts like the ones I mentioned that may be guiding you.

KEVIN: I don't know if this is one, but I always think I'm a loser because I'm not as good in sports as my brother.

LEADER: That would be one. Others of you?

VICKI: Mine doesn't have anything to do with sports. Mine is with grades. I'm dumb because I can't make all As and my mom and dad go bananas over my two brothers' grades.

ED: My mom tells me I'm lazy and worthless all the time, especially when she's stressed out about work or some boyfriend.

LEADER: Ed, what would you say your guiding fiction is about what your mom says?

ED: I always feel bad because Mom's always cutting me down.

LEADER: What do you think about yourself?

ED: Something's wrong with me because I can never please Mom.

LEADER: Do others of you tell yourself something is wrong with you?

VIARA: I always feel something is wrong with me. I'm never good enough and never will be—that's what I believe about myself.

LEADER: Let's focus on these ideas. Do you see how they cause you to feel bad about yourself?

VICKI: I can see it in others but not me. I am dumb. I am terrible in math.

LEADER: Vicki, that's a guiding fiction that you can challenge because it is not true. For one thing, I know you are not terrible in math. I believe you make Cs and Bs in math. Let me say again, a guiding fiction is something that guides you but is not true. I want each of you to either comment on your guiding fiction or on one of the other members'.

ED: Vicki, because you aren't as smart as your brothers or not good in math sure doesn't make you dumb. I know you are a great reader and you've helped me in science.

LEADER: What do you think about what Ed is saying? Can you see how this mistaken thought guides you and causes you to feel bad?

VICKI: *(With a sad voice)* But Ed's not my mom or dad.

LEADER: Vicki, we're looking at whether your thought about being dumb is fiction. Is it true or not that you are dumb? How many of you would say it is not true? *(Everyone raises his or her hand.)*

In this example, the leader would continue to explore the different guiding fictions and try to get members to see the fictional part of their guiding beliefs. The leader would continuously challenge the members to look at thoughts that are guiding them.

■ ■ ■

EXAMPLE 4

This is the third session of a group of college freshmen.

LEADER: I want to talk about something that may shed some insight into why you might be anxious. When growing up, most people form some mistaken goals that often stay with them for life. Mistaken goals are goals that you have based upon some faulty logic or thinking. A mistaken goal might be *I must get good grades to prove to the world that I'm not a dummy.* Or *I must do better than my sibling so that I'll remain my parent's favorite.* Another may be *I must become a doctor or lawyer or teacher or whatever in order to be loved.* What I'm saying is that you may be driven by some mistaken goal that formed during your childhood that you are unaware of. Does this trigger anything for anyone?

WAYMAN: I know I want to do well because all my life I heard I was the lazy one just like my dad. I don't want to be like him—he's a homeless alcoholic. Everyone thinks I'll fail. I worry about it all the time. That's why I joined this group.

LEADER: *(To the group)* So what would be Wayman's mistaken goal?

GAIL: To quit worrying?

LEADER: That's what he wants to do but his mistaken goal would be what?

KAHIL: Isn't it that he has to succeed to show people he's okay? I thought last time we talked about how we are all okay and need to accept ourselves.

LEADER: That's right. The mistaken goal would be *I must do well to prove that I'm okay and that I won't be like my alcoholic dad.* Wayman, it's good that you want to succeed but hopefully it will be for you and not to simply prove these people wrong. You can make the decision to not be like your dad and that has nothing to do with making it here at college. This is a good example of a mistaken goal. We'll come back to this in a few minutes, Wayman. You think about it. I just want to hear from others and then we'll discuss different people's mistaken goals. How about others of you?

VANN: I have to become a doctor because my sister is a dentist and I could never live with myself if she made more money than me. Also at family reunions I know people would favor her over me. Now they make a big deal over her being a dentist and I want them to make a big deal over me.

IAN: Do you want to be a doctor?

VANN: What do you mean? I don't have a choice.

LEADER: What do you think Vann's mistaken goal is?

The leader would focus the group on helping members identify and challenge their mistaken goals and then help members see how life can be a series of choices rather than striving to reach some mistaken goals that are based on early messages.

EXAMPLE 5

It is 15 minutes into the fourth session of a divorce-recovery group for people who have been divorced for 6 months or more.

HILDA: I just feel so off balance. I don't do anything and I don't go anywhere. My life is awful.

LEADER: Let me pick up on that and talk about ways to make all of your lives better. *(To the entire group)* I want to get away from talking just about your marriage and divorce. This psychiatrist named Alfred Adler describes three important life tasks that appear to be universal and must be dealt with. They are social relationships—family and friends, love relationships, and work. No doubt one of these life tasks is out of whack for all of you—love relationship—but let's look at the other two. My concern is that you are not putting any effort into the other two tasks, and with your love task being at a low point, it is no wonder you feel bad and off balance. What are you doing in the other two tasks?

ROGER: What do you mean about work? I go to work but it's not the same. I hardly talk to anyone and I don't enjoy it like I used to.

LEADER: Enjoying work is what this refers to. Ideally, a person enjoys the work he or she does and I know all of you have jobs that you can enjoy.

MARQUITA: Since the divorce, I just go through the motions of teaching, and it's really not fair to those first-graders.

BARB: I used to love going to work at the bank. I loved talking to the customers and the other tellers. Now, I just go and put in the time.

LEADER: This is exactly why I brought the topic up. You can't do anything about the divorce but for sure you can do something about your attitude at work and that can help you feel more positive about life. Just because your love life is not going well does not mean you can't enjoy work. In fact, it is much better if you do try to enjoy work, even if you have to fake it some. That is, fake it 'til you make it. (*Members laugh.*) I want us to focus on work and how to enjoy it more and then we'll look at the other life task of family and friends.

DARLA: That'll be good. Friends are begging me to do things but I tell them I'm not ready.

BARB: I've completely shut out my family. (*Others nod their heads.*)

LEADER: We'll come back to family and friends, and I can see that most of you are neglecting that life task as well. Let's focus right now on work. What can you do to make your work more enjoyable?

The leader would focus this session on these two life tasks because they are important to a balanced life and then would follow up each week on how people are doing in these areas. Using the Adlerian concept of life tasks often can be helpful in many different kinds of groups.



GROUP COUNSELING SKILLS: Adlerian Parenting Principles

Watch video 17.3—a psychoeducational group using Adlerian principles in a parenting group. The leader shows a variety of leadership skills in the middle and closing phases of the session.

■ ■ ■

Suggestions for More Reading

Sonstegard and Bitter's (2004) book is solely about using Adlerian theory in group counseling and therapy situations. The book is titled *Adlerian Group Counseling and Therapy.*

Transactional Analysis

TA is a comprehensive theory of human behavior developed by California psychiatrist Eric Berne and is a dominant theory in Europe, Australia, and New Zealand. TA theory has many important and valuable concepts, such as scripts, strokes, and games that can be useful in groups. In this section we focus primarily on the TA concept of ego states and scripts because they can be easily taught to group members. TA is good for group counseling because it is a very visual theory and members can easily relate to it.

According to TA theory, everyone has three ego states: the Parent, Adult, and Child. These ego states are developed as a result of childhood "messages." Messages such as, "You are a bad child" or "Only stupid children make mistakes" or "Don't trust anyone," may come through direct comments from parents or other caretakers or through early life experience. There are two parts to the Parent Ego State: the Critical Parent and the Nurturing Parent. The Child Ego State also has two parts: the Natural, Free, or OK Child and the Adapted or Not OK Child.

Which ego states are dominant within a person depends on a person's experiences. For example, Julie is a person who was raised by a very critical mother. Regardless of how Julie tried to please, her mother found fault. As Julie grew into her preteen and teenage years, she incorporated many of the negative comments her mother made to her about everything from her appearance to her

friendships. As a young adult, Julie used her mother's judgments as if they were her own beliefs. Julie *internalized* her mother's critical comments and developed her own Critical Parent Ego State. Now Julie criticizes herself for making common mistakes and often gets very angry at these errors. She also is critical of others and at life in general when things don't go well for her. Because Julie's mother did not deliver many nurturing messages, Julie has difficulty nurturing herself or others.

As a very young child, Julie felt the pressure to conform to her mother's demands. Therefore, while Julie was developing her Critical Parent, she was also developing an Adapted Child Ego State, seeking approval of others, fearing disapproval, and conforming or adapting to situations to avoid criticism. Now Julie is living her life using her Critical Parent Ego State and her Adapted Child Ego State, with perhaps less Natural Child and Nurturing Parent. Most importantly, however, is the fact that Julie does not use her Adult Ego State, especially in interpersonal decisions, and seeks out relationships that are ultimately critical and abusive. In a group, the leader who uses this theory would try to help Julie become aware of her different ego states and how each ego state influences her decisions and interactions.

Teaching TA to Group Members

TA can be taught by the group leader in mini-lecture format, but we feel that a better way to introduce TA is by using a member's problem or situation. The following example shows one way to introduce and teach TA to the members. It is the second session.

Leader: Many of you have mentioned fighting or arguing with different people in your lives. I think this is a good time to introduce some tools that can help you understand that better. Tonight we're going to discuss Transactional Analysis, or TA. Many people have found these ideas very helpful and I think you will too. Julie, if it is okay with you I thought we might use your example with your boyfriend.

Julie: That's fine with me. I want to stop fighting with him.

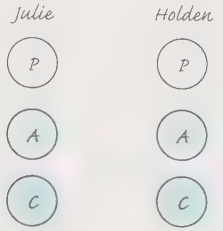
Leader: Give us an example of a typical fight.

Julie: I like some attention on holidays and birthdays. He often waits until the last minute or even forgets. Like last week, it was our 1-year anniversary of dating and he didn't get me anything. We got in a big fight. I yelled at him and he yelled back and then I started crying.

Leader: That's a good example. *(Leader goes to whiteboard and draws as he talks.)* In each of us are three parts or ego states: the Parent, Adult, and the Child. *(Draws three circles.)* So Julie, you have these three parts and your boyfriend has these three parts. What is his name?

JULIE: Holden.

LEADER: *(Draws three circles under Holden's name. Glances at members and sees them all looking at the whiteboard.)*



LEADER: The Parent part is the part of you that is either nurturing or critical, or the part that has all the *shoulds* for yourself and for others. In your case, would you say you are more critical or nurturing in the relationship?

JULIE: Critical, most definitely. I'm critical of him, of me, of everything. I'm not as bad as my mom, but I'm afraid I'll get like her as I get older.

LEADER: All of you, once you understand this model, you will have a much better understanding of yourself and of others, and you can make a choice as to what ego state you want to be in. Let's now go to the Child part. That ego state actually has two parts—the Fun Child and the Hurt, Angry, or Pleasing Child.

ZENA: I get my feelings hurt all the time—is that the Child part?

ROSE: Me too, and then I get real mad.

LEADER: You're right. You would be in the Child ego state. Julie, how about you?

JULIE: I'm all over the place. Often I'm hurt and then I get real mad and throw a fit, which he hates because I break things and then there are times when I go all out to please him. I'm a mess.

LEADER: I think understanding your ego states will make a great difference. I want to teach about the Adult and then we'll spend the rest of the session talking about how each of you can apply this to your lives. The Adult is the thinking part—the rational part. Like right now, I think most of you are in your Adult part listening to me and thinking about your own situations. I hope none of you are in your critical part, beating up on yourself as I describe the ego states.

CORRIE: Gosh, that's exactly what I was starting to do because I realize I'm so critical of my boyfriend because of the way I was raised.

He's always said it is my fault that we fight and now I see maybe he is right.

LEADER: Corrie, I hope you can use your Nurturing Parent and Adult part today because beating up on yourself certainly will not change things, but learning this will.

CORRIE: I knew I was messing up but I didn't know how. I think this might help me.

LEADER: Let me say a little more about the Adult. If you stay in the Adult part, you'll find you will rarely, if ever, fight with someone because you are thinking and understanding the situation, and you are not raising your voice or feeling hurt or trying to win. You are thinking. Most fighting happens when people are in either their Child or Critical Parent or both or are switching back and forth between the two, but they are not in their Adult.

JULIE: That's me most of the time—hurt and critical and not thinking. My boyfriend even says to me, Julie, stop and think for a minute.

LEADER: We'll work on that in a few minutes. Let me get some general reactions to this. Let's talk about anything you are thinking about as you look at yourself through this model. This applies to just yourself or your interactions with others like boyfriends, friends, bosses, or family members.

The leader would then have a discussion about the different ego states. He would try to get members to focus on some of their specific concerns using this theory. Once he felt the members understood the ego states, he would try to get members to see how the Adult functions differently from the Child in situations, such as the ones Julie or Corrie were describing. The key in TA is to help members get an Adult perspective on situations that tend to be negative or volatile. Often the leader or the members use the whiteboard to draw the specific interactions that are being described. If you are not familiar with this theory, we encourage you to seek out additional reading.

Using TA in Groups

Once the leader teaches TA, she can use the Parent, Adult, Child concepts in numerous ways when working with one member or with the entire group. One reason TA is an excellent theory to use in groups is that it can be used in a variety of visual and experiential ways. The leader can:

1. Draw PAC circles in various sizes with arrows going from the different ego states on a whiteboard to represent an individual or interactions being described by the working member.
2. Use chairs to represent the ego states.
3. Act out the scene and then have members play the different ego states.

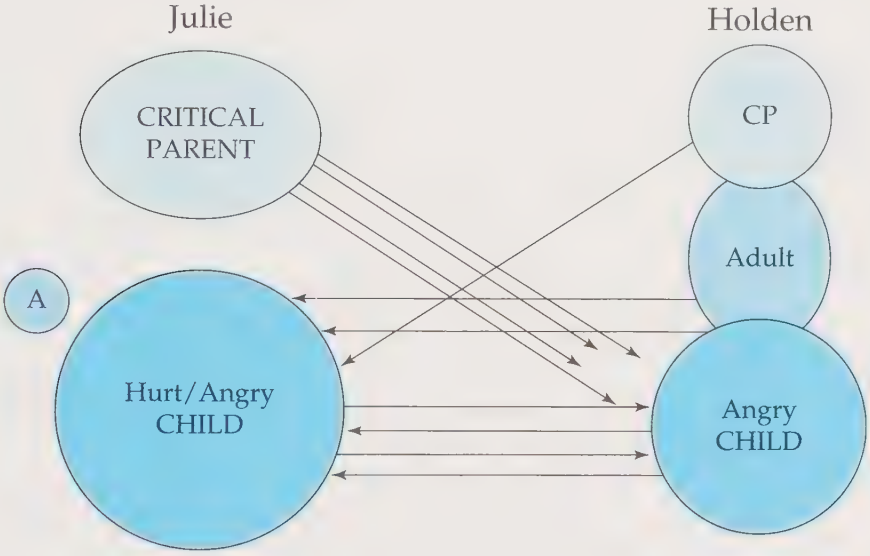
EXAMPLE 1: Drawings of Interaction

This continues the example of Julie from above.

LEADER: Julie, I want to focus on the transactions that you and your boyfriend have. It sounds to me like you interact during the fights from either the Critical Parent or Hurt/Angry Child. What ego state does he come from?

JULIE: He stays calm for a while and then he blows up and starts putting me down, and then we end up yelling at each other.

LEADER: Let me draw this for you. I also think others of you can benefit from seeing this.



The leader would explain the drawing and get Julie and other members' reactions about how they often fight from their Critical Parent and Angry Child and how they may have a small Adult sitting on the sidelines. Often the drawing helps members better see what they are doing that fuels the fighting in their relationships.

EXAMPLE 2: Drawing of Different Size Ego States

It is the second session of a parenting group. The leader taught the members TA in the first session. It is 10 minutes into the session, and members are sharing about how they tried some of the things discussed in the first session. Four have shared, and Maxine and Paula are left to share.

MAXINE: When you said last week that you thought the TA model was one of the best tools you've ever learned for helping parents and how it absolutely can change the way we parent, I thought you were exaggerating, but you were right. I've been paying attention to which ego state I'm in and stopping myself from going to the Critical Parent. I'm amazed at how much I was in my Critical Parent. No wonder there was so much yelling and fighting between me and the kids.

PAULA: I did good the first couple of days but then Wilson made me yell at him. He knows I have to leave for work by 7:30 and he was watching TV at 7:15 and was not dressed. He knows better—that's what gets me. And then my little one, who is 4—that's a whole other story. At the doctor's office we had to wait for over 45 minutes, and he would not sit still. It was embarrassing that he made me yell at him in the waiting room.

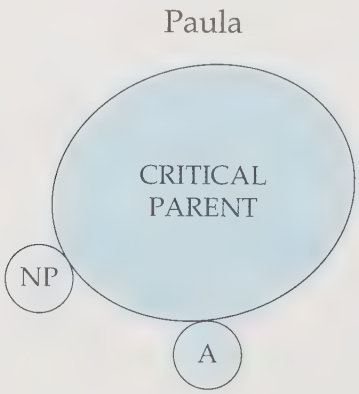
LEADER: Paula, would you like to understand what happened in those two situations?

PAULA: I'm here because I don't want to be like my mom. I don't yell all the time like she did but I can't stop myself sometimes.

LEADER: *(In a kind, caring voice)* We can be of help and I do want to say this to all of you—no one makes you yell at him or her. That is your choice. You choose to yell, and the TA model can help you with this.

ROBERTA: *(To Paula)* I did something I've never done before. I stopped myself almost in mid-sentence. I went into my daughter's room and started to yell at her about the room and for some reason I thought about the PAC model we talked about and I stopped after just a couple of words and then used the nurturing part. Oh, what a difference! We didn't fight. We actually spent the next 20 minutes talking while we together picked up her room. It was great.

LEADER: Paula, let me show you something that may help. From what you described, your Critical Parent is much larger than your Adult part. *(Draws circles—everyone looks at the drawing.)*



PAULA: That's right. My mom was worse. Can I ever get my Adult and Nurturing Parent to be larger than my Critical Parent?

ROBERTA: Let me jump in here again if I may. I just realized that I was able to have my Adult and Nurturing Parent be in charge with my daughter this week, but boy, I'm just like the drawing when it comes to my husband. I lost it with him.

LEADER: For all of you, it is possible to get your Adult and Nurturing Parent to be in charge, but that's why we are meeting for eight weeks. It takes practice, awareness, hard work, and determination to change. Paula, you can do this if you pay attention and not do what is automatic, because automatic to you is yelling, given what you've told us about your mom.

PAULA: I don't know how my Adult is supposed to think.

LEADER: *(To group)* Any ideas?

MAXINE: What about realizing that your kids are kids? No 4-year-old can sit still for 45 minutes. I can't tell you how many times I thought about that Adult phrase we learned last week: "Get your expectations in line with reality." That's helped me so much. Paula, do you have your expectations in line with reality regarding Wilson and your little one?

PAULA: Not really. I just expect them to do what I want them to do.

MARLA: I just realized I don't either. I want my kids to act like I did, which was to always be good. But I was scared to death not to be. My kids aren't afraid of me so they are going to act differently. That's the Adult perspective, right?

LEADER: Most definitely. In this group we are going to try to strengthen the Adult part by giving you information and letting you share thoughts and ideas. The goal is to gain an Adult perspective on a situation and hopefully shrink the Critical Parent. One of the best things you can do is to get rid of most of the "shoulds" you have for your children and get an Adult perspective on how they are. Paula, let's take Wilson in the morning. What does he do and how can you better understand him?

PAULA: But he should get dressed before he watches TV or whatever!

ROBERTA: That's the Critical Parent part, right?

LEADER: Right. Getting your expectations in line with reality will create a much calmer morning than having "shoulds" for Wilson.

PAULA: That's the problem. I think I'm getting it. I've got to understand better how Wilson is in the morning and help him get himself ready instead of yelling at him.

LEADER: Say some more. That's coming right from your Adult.

■ ■ ■

EXAMPLE 3: Use of Chairs

This is a group for getting along with ex-spouses. The members were taught TA last week and are now discussing issues that came up during the current week.

LEADER: All of you can complain or you can do something about it.

IVAN: I am afraid she'll not let me see the kids if I say something to her. Last time I did, we had a huge fight.

LEADER: *(Pulling out three chairs to the center of the group—one chair is a small child's chair.)* Which chair were you in the last time you fought—was it from the Critical Parent *(Leader stands on one of the chairs)*, was it from the Angry Child *(Leader sits in the child's chair)*, or was it from the Adult *(Leader puts a sign that says THINK in the chair representing the Adult)*?

IVAN: Well, uh, it was not from the Adult chair.

LEADER: All of you look at the three chairs. Which seat are you in when you interact with your ex? From what you said last week, no doubt some of you have difficult ex-spouses, but what we can do here is get you to look at what ego state you come from when you interact with them.

PHYLLIS: I start in my Adult and then I end up in the Critical Parent chair. *(She stands on the chair.)* I blasted him before we got divorced and I still do. It makes no sense.

LEADER: I'd like to teach all of you how to stay in the Adult. Are you ready to learn how to stay in the Adult chair? *(Everyone is staring at the chairs in the center.)*

The leader then leads a discussion using the chairs, having members sit or stand in the various ones as they describe their interactions with their ex.



GROUP COUNSELING SKILLS: Theories - TA

Review video 10.2 and watch how the leader uses the small chair and the Adult chair.

■ ■ ■

EXAMPLE 4: Acting Out the Ego States

It is a changing families group for eighth-graders. It's the third session and the members have learned TA.

LEADER: Because many of you have said you fight with your parent or step-parent, let's act out a couple of situations. Tommy, what about the one you were describing with your stepdad about homework? Are you willing to have us act that out?

Tommy: Okay. I'd like to see how to get him off my back!

Leader: He comes into your room and offers to help you with your math or science, right?

Tommy: *(Angrily)* Yeah, what business is it of his how I do? I'm not his kid.

Leader: *(To group)* Listen carefully because I'm going to ask one of you to play Tommy and someone to play his stepdad. Are you saying that he enters in an upbeat, helpful mood and you bite his head off? What does he end up doing?

Tommy: Yeah, he's in a decent mood and leaves pissed off, slams the door.

Leader: Who wants to play Tommy and who wants to play his stepdad? *(Two volunteer.)* I want you to be studying and I want you to come into his space and ask if you can help. Toby, act like you are reading or something.

Cliff: *(Acting as the stepdad, using a kind voice)* Tommy, need any help? I've got some time.

Toby: *(Acting as Tommy with a gruff voice)* No! I told you I would ask you if I needed any! Quit bothering me.

Cliff: Look, I'm just trying to be helpful. I know you can go to college if you put a little effort into your studies.

Toby: It's none of your business whether I go to college!

Leader: *(Stopping the role-play)* Tommy, is this close to how it goes?

Tommy: Yeah. You guys are good.

Leader: They did do a good job because you have done a good job describing the scene. What do you see in this role-play? What ego state is he in?

Cliff: He just jumped down his stepdad's throat. He's in his Angry, Hurt Child, I would think.

Leader: Do others agree? *(Heads nod.)*

Tommy: How do I get out of my Child? What should I do?

Leader: We can show you in a minute, but first can you tell us why your Child reacts to him like you do?

Tommy: Well, uh, *(tearing up)* he's not my dad. I miss my dad and when Mom married my stepdad that pretty much meant that my mom and dad weren't going to get back together.

Leader: So you take it out on him?

Tommy: Yeah, I guess that's right.

Lonny: I'd like to say something. Watching this was real helpful. I am completely in my Child with my stepmom.

Toby: Playing the Child part sure helped me to see what I do sometimes. Are you going to help us stop acting the way we do?

LEADER: I'm going to try to make this group a place where you can help yourselves like you are doing now.

CLIFF: I like this TA stuff and this role-play stuff.

■ ■ ■

Other Aspects of TA

In this section we just touch on some of the many theoretical aspects of TA. Group leaders who use TA often discuss members' life scripts, egograms, stroke economy, or games that they play with significant others. For those interested in more information about TA in groups, an excellent place to start is the Web page ta-tutor.com. This Web page has 400 handouts you can download. Also, *TA Today* by Stewart and Joines (2012) is a good introductory book on Transactional Analysis. On YouTube there are many good clips that explain TA. In *Impact Therapy: The Courage to Counsel* (Jacobs & Schimmel, 2013), there are many examples of using TA in multisensory ways.

Gestalt Therapy

Fritz and Laura Perls developed Gestalt therapy in the 1940s. Gestalt therapy, an existential approach, focuses on creating a therapeutic environment in which the client gets "in touch" with unfinished issues by "presentizing," re-experiencing, and integrating such experiences into current awareness. Regardless of past experiences, what is important is the client's interpretation of those events in the here and now. Gestalt theory recognizes that clients tend to compartmentalize their lives, "hiding" disowned or unacceptable parts from the conscious self. The ultimate goal of therapy is the integration of these parts and movement toward independence, maturity, and self-actualization. While many approaches to therapy had relied heavily upon the therapist's interpretation, the Perls promoted the concept that only the client can effectively interpret his awareness (Strumpfel & Goldman, 2002).

Fritz Perls introduced the concept of the "hot seat," a voluntary experience where the therapeutic focus is on a single member while other members watch. He also adopted the "empty-chair" technique, whereby a chair is used as a prop. For example, the client may be asked to imagine a person in the chair and speak to that person and then switch chairs and play that person in response. Many therapists use Gestalt techniques in conjunction with other theoretical models. In group settings, exercises and activities, such as guided fantasies, are used to generate awareness and create a here-and-now experience. Using Gestalt techniques requires a thorough knowledge of each technique and the skill to apply it. Because awareness occurs in the present moment, considerable psychological pain may result as members delve into feelings and recount difficult, perhaps

traumatic, experiences from the past. Therefore, leaders must not only have the knowledge and skill, but also be prepared to deal with the intense emotions that may be generated.

Using Gestalt Therapy

In this section, we offer a number of examples showing how Gestalt techniques can be used in various group situations to heighten members' awareness.

EXAMPLE 1: Putting Someone in the Empty Chair

Betty is a young woman whose parents died several years earlier in an auto crash while on a driving vacation. Betty has brought this up before and has started once again to talk about her parents.

BETTY: *(Crying)* I don't think I can make it without them.

LEADER: Betty, do you want to try to finish this unfinished business?

BETTY: I don't know how.

LEADER: Are you willing to try? I've got some ideas. Do you want us to try to help you with this?

BETTY: I have to do something. I don't want to feel this way forever.

LEADER: Okay, let's do some work. *(The leader sets up two chairs in the center of the group facing the member, one to represent her mother, one her father.)* Which parent do you wish to address first?

BETTY: My mother.

LEADER: *(The member is looking at the leader. The leader, using her hand to direct the member's attention to the "mother chair," speaks in a gentle voice.)* What would you like to say to her?

BETTY: *(The member shifts, stares at the seat of the empty chair and tears up.)* I miss you so. Why did you leave me? You know I need you. Did you have to go on that trip with Dad? I know you really didn't want to go. *(The member's voice rises in pitch toward the end of her comments, suggesting feelings of anger.)*

LEADER: How are you feeling?

BETTY: Sad.

LEADER: What else? *(The member gazes at the leader, as if not quite ready to identify her feeling. The leader's voice, while kind, is more intense.)* I noticed your tone changed, as if perhaps you are feeling some anger toward your mother for leaving you, for going on this trip she didn't want to go on and not coming back?

BETTY: *(Sobbing)* How can I be angry at her? She's dead. *(In Gestalt theory, the member has compartmentalized her feelings of anger and denied them.)*

LEADER: *(Soft, reassuring voice)* It's okay to be angry at someone we love. If you sit on your anger, it will be difficult to put this behind you. *(The leader again uses a gesture to direct the member's attention to the empty chair.)*

BETTY: *(Sobbing, in a loud, angry voice)* Why did you go? Why did you die and leave me all alone? You know how hard it is to live without your help!

LEADER: *(The leader scans the group briefly, observing that the members appear very involved.)* What do you need from your mother in order to let her go?

BETTY: I want to know that she loves me and that she didn't mean to leave me alone.

LEADER: *(Gesturing to the empty chair)* I'm going to ask you to sit over there, in that chair. *(The member moves to the chair representing her mother.)* I'd like you to be your mother. What do you have to say to Betty?

BETTY: *(Having changed seats, she speaks as her mother.)* I do love you and I didn't mean to leave you. It was an accident, something we couldn't avoid. It wasn't something I did on purpose. I would not have left you on purpose.

The leader directs the "dialogue" between Betty and her mother for several minutes, having the member switch seats until she is able to realize that her mom did not "do this to her" and that her anger is unfounded. In this example, the group leader uses the empty chair to help the member *presentize* her feelings toward her mother, state and release those feelings in an acceptable way, and integrate this new awareness. When Betty seems finished, the leader turns to the group.

LEADER: What did Betty's work mean to you?

DEION: This blew me away. Betty, I was right with you. I've been mad at my dad for his drinking and this helped me to realize that he's not doing this to me. This was very powerful. Can I talk about this?

LEADER: Sure. Let's hear from others and then, Deion, you can do whatever work you need to.

ERIC: I just kept picturing my grandfather in the chair and thinking of all kinds of things I want to say to him.

Many who are not familiar with Gestalt Therapy often comment on how difficult they believe it will be to ask a member to talk to an empty chair and even move into that chair to play another person or another part. As you read through the various examples, we hope it is apparent that the working member is so involved in the experience, she follows the leader's directives. There are occasions when a working member may have trouble getting into the idea of talking to an empty chair because the member is experiencing some discomfort for one reason or another. The following dialogue, using the same situation as before, shows how the leader would handle such a situation.

LEADER: *(After carefully setting the stage with Betty)* What would you like to say to your mother?

BETTY: I don't know.

LEADER: *(Looking at the empty chair, rather than at the member)* You said you had some feelings toward your mother. What can you say to her about those feelings?

BETTY: I don't know what to say.

LEADER: *(Still focusing on the empty chair)* Mom, I have some feelings that I just can't get out. *(Turning to the member)* Try that.

BETTY: Mom, I have some feelings I just can't get out.

LEADER: *(Again, speaking to "Mom" in the empty chair)* "One feeling is...." Go ahead.

BETTY: One feeling is .... anger. *(She sobs.)*

Here the leader knew she had set up the therapeutic situation correctly and could see that the member was not bringing her mom into the present. By pushing the member gently and playing the member and using the sentence completion, she helped the member to get into the experience. If the member had continued to not be able to "put her mom in the chair," the leader could drop the use of the chairs, pair the members up, pairing herself with the working member, and discuss one-on-one what the member was experiencing. The value of the empty chair is that it is so powerful that even when only one member is in the "hot seat," most members are thinking about someone they would have in the hot seat.

■ ■ ■

■ EXAMPLE 2: Inner Dialogue

In the fifth session of a counseling group, members are discussing love relationships and marriage. One member has been unable to make a decision about her marriage. In this example, the leader uses an empty chair to facilitate an *inner dialogue*.

LEADER: This has been a struggle for you, Sally, trying to decide whether you want to leave your husband or not.

SALLY: There is a part of me that does, and a part that doesn't.

LEADER: So it seems as if there are two parts with opposing views.

SALLY: Yes. And depending on which one is running things, that's how I feel. I try to make a decision, but just when I think I've made it, the other part takes over.

LEADER: What would you think about having the two parts talk it out? Would you like to try a dialogue? *(In anticipation, the leader pulls up an empty chair.)* You could start in one part and say what you'd like to the other part in that chair.

SALLY: Where do I begin?

LEADER: You choose. Start as either the part that wants to leave or the part that wants to stay. Others of you probably will benefit from watching this because you have two parts to an issue.

SALLY: I'll do the part that wants to leave first. I just don't want to be married to him any more. He is abusive and I know he's had affairs and is probably having one now. I don't love or respect him and I'm not happy. We just go through the motions. It doesn't make sense.

LEADER: Now switch chairs. Sit in the opposite chair and take the other side. *(Leader sees that other members are watching very intently.)*

SALLY: *(Switches chairs; the leader observes the change in the member's posture. She now slumps and looks down as she speaks.)* But in his own way, he loves me and he's a good provider. Besides, I've grown to love his family.

LEADER: Now switch chairs. Tell the part that wants to stay why you need to leave.

SALLY: *(Switches to the first chair: Again, her posture is more upright, her voice firmer.)* It's over. There is no reason to stay.

LEADER: Ask her what she is afraid of.

SALLY: What are you afraid of? *(Switches seats)*

SALLY: *(The member tears up, speaks in a little girl's voice.)* I'll be alone. He takes care of me. I don't know what I'll do if I'm on my own. I haven't worked in 14 years.

LEADER: Switch back to this chair and again be the part who wants to leave. *(She does.)* Does this help others of you? *(Many nod.)* I'll get your comments in a minute.

SALLY: *(Pointing to the other chair)* She's hanging on. She is really afraid of being alone. And I think she is stronger. I'd like to strengthen this part so I can free myself up to make this decision.

■ ■ ■

■ EXAMPLE 3: Empty Chair—Role Reversal

This is the fourth session with five high school students meeting to discuss conflicts with adults, particularly parents and teachers. Bill has been discussing problems with his father.

LEADER: So, Bill, can you tell us more about your dad?

BILL: He's a royal pain. He's gone all the time. When he comes home, all he does is bitch and complain. I'm tired of it.

LEADER: He complains about you, about the family? What exactly?

BILL: Everything.

LEADER: Can you be your dad? I mean, can you be that complaining man we've heard you talk about so that we have a better idea of what you have to put up with?

BILL: *(Sneers and shuffles his feet.)* Sure. What, right here? Just be him?

LEADER: Tell you what. Here is a fresh chair. *(The leader moves a chair into the circle next to the member.)* Move into that chair and be your dad. And I'll interview you, kind of probe you to get you to tell us more about yourself. *(The leader is purposefully structuring the role reversal to tap into the information he wishes to draw out of the working member. The other members are paying close attention.)*

BILL: *(Body language communicates reluctance as he moves to the next chair. He sits with his legs and arms crossed.)* Okay, shoot. What do you want to ask me?

LEADER: Mr. Adams, Bill tells me you're gone a lot and when you come home, all you do is complain.

BILL: *(As the dad)* That little pipsqueak! He would talk about me behind my back. Like he ever contributes anything to the house. I'm out busting my ass, and all I get is lack of respect. You think I like working two jobs? I'm tired when I get home. Then I got to do half the things he was supposed to do.

LEADER: *(The leader sees that Bill has quickly gotten into the role of his dad.)* So you feel you have a right to complain.

BILL: *(As Dad)* Damn right. He goofs off in school while I'm out there trying to make ends meet. You think I don't want to be home more?

LEADER: Are you saying you are concerned about your family?

BILL: *(As Dad, hesitantly)* Yeah. Yeah, I guess that's what I'm saying.

LEADER: Bill, sit here, be yourself. *(Bill moves to his original chair.)* What are you learning from doing this?

BILL: *(Slowly and thoughtfully)* Well, I never thought about what it's like to live my dad's life. I guess a lot of what he does he does for us. I just wish he wouldn't complain about it so much.

LEADER: *(Having taught TA last week, he uses that language because it seems to fit well with this situation.)* So he's in his Critical Parent a lot? *(Member nods.)* And you go to your Not-Okay Child rather than getting into your Adult? *(The member nods and smiles in recognition. The leader glances around the group.)* Does anyone feel he could play Bill's dad so Bill can get a chance at practicing staying in his Adult?

DON: I think I can play his dad.

EXAMPLE 4: Bringing the Past into the Present

In this fifth group session of sexually abused women, the leader has conducted a fantasy exercise, guiding the members back into memories of childhood. One member is beginning to experience what she perceives as the "split" between the "good girl" part of her and the "bad girl," or abused part. The group leader is carefully encouraging this new awareness.

■ ■ ■

LEADER: Chandra, tell us about the good girl.

CHANDRA: *(Gazing off as if she is visualizing an image.)* She is very good.

LEADER: And what is the good girl doing?

CHANDRA: She is playing with her cousins in the front yard, having fun and then my 16-year-old cousin asks me to go with him.

LEADER: What are you feeling as you go with him?

CHANDRA: Scared. Wanting someone to help. Not wanting to go but not able to say no because I try to please everyone.

LEADER: And what do you see?

CHANDRA: Me being a bad girl and him threatening me.

LEADER: Now what is happening?

CHANDRA: *(Suddenly she sobs uncontrollably, then shudders and screams. She rocks back and forth in her chair.)* Why couldn't someone help me!! Why didn't I do something to stop it!!

LEADER: *(The leader motions to the member next to Chandra to relinquish her chair so the leader may be in closer proximity to offer her strength and support and also help the member come back from the pain she is experiencing.)* Chandra, I want you to hear my voice. We need to look at your pain now. That was many years ago. I want you to look up, look around. *(Chandra stifles a sob and looks around the group.)* There are people here who want to help you finish this unfinished business. *(Chandra nods.)* I want you to switch seats and sit in this chair, today's chair, and talk to the little girl who was abused in 1988. Others of you may be thinking if you are stuck in the past. *(A number of head nods.)*

This leader has the knowledge and courage to not be "freaked out" by her member's reaction. The leader stays with Chandra psychologically and helps her work through painful material. This ability is essential when using techniques that have the potential to deepen a member's awareness of past trauma. The leader must also be tuned into other members who may have past memories of abuse that could be triggered by Chandra's work.

■ ■ ■

EXAMPLE 5: Use of Specific Language

In Passons's book on Gestalt approaches, he points out that the specific use of words can block our awareness and prevent us from making alternative choices (Passons, 1975). For example, clients may refer to "it" rather than "I" as a way of avoiding responsibility for their feelings and motivation. Some common statements that members might make are, "It's depressing to lose a job," rather than, "I'm depressing myself regarding losing my job." Or, "It's going to be impossible to get over her," rather than, "I make it hard to get over her." Or "It's scary in this group," rather than, "I scare myself about being in this group." By using *it* or *you* rather than *I,* we tend to depersonalize our feelings and avoid responsibility. Another language barrier to becoming more aware of feelings involves the use of "can't" statements, rather than "won't."

In the following example, Odetta is dealing with a fear of failure that holds her in a low-paying job.

ODETTA: ... I hate my job but I can't quit my job.

LEADER: Odetta, try this. Say, "I *can't* quit my job." Then say, "I *won't* quit my job."

ODETTA: I can't quit my job. I won't quit my job.

LEADER: Which is more accurate?

ODETTA: I guess that I won't or chose not to quit. I'm afraid of the unknown.

LEADER: Do others of you use the word *can't* to avoid taking responsibility for your choice? *(Heads nod.)* Any of you want to comment?

HENRIQUE: I always say I can't lose weight and I can't get up early to exercise.

LEADER: Let's spend some time on how our language affects us.

■ ■ ■

EXAMPLE 6: Focusing on Nonverbal Behavior

Perls recognized that members' nonverbal behavior reveals much about how they are feeling. In the following example, a member discusses the loss of his job.

LEADER: Ted, it sounds as if you were really surprised when your boss just let you go without any warning.

TED: *(Laughs)* Sure was. Boy, you just never know. I had that job for 3 years and, bingo, I'm out.

LEADER: Some people might feel angry under these circumstances.

TED: *(He smiles.)* Yeah, I'm angry.

LEADER: Are you aware that you are smiling even though you say you are angry?

TED: I said I'm angry. *(Smiling a tighter smile.)*

LEADER: *(Looks around the group and chooses a member he believes will respond well to his request.)* Jack, could you show Ted how he looks right now? Ted, just watch for a minute and tell us your reaction. *(Jack imitates Ted's smile.)* What is the smile saying?

TED: Friendly, safe.

LEADER: I'm going to ask you to try something. I'm going to ask you to be your friendly, safe smile. Will you do this?

TED: Like this? *(He forces a smile.)*

LEADER: That's right. Now I'm going to ask you to talk to us *as* your smile. Something like this. "I'm Ted's smile and I serve a good purpose for him when he's angry. The purpose I serve is . . ."

TED: I'm Ted's smile and I serve a good purpose for him when he's angry. The purpose I serve is to keep me from the rage I feel.

HAVIER: What is that about?

TED: My dad was a true rageaholic and I made a pledge to never act like him.

LEADER: Would it be helpful to express some of that anger here rather than keeping it in with a smile?

FRANCIS: I laugh to cover up my true feelings. This is good for me to watch.

LEADER: Let's finish with Ted and then maybe we can focus on that, Francis.

■ ■ ■

Cautions Regarding Gestalt Techniques

It is important to understand that Gestalt techniques, more so than most other techniques, can tap quickly into deeper feelings. Members may experience painful or traumatic memories and emotions. Therefore, if you choose to use Gestalt techniques, you want to make sure that you are prepared to deal with deeper emotion.

Suggestions for More Reading

Gestalt Approaches in Counseling (Passons, 1975) is an excellent book for understanding different Gestalt techniques. *Gestalt Therapy Verbatim* (Perls, 1969) is an excellent book to read to find out how Fritz Perls led his "hot seat" Gestalt groups.

Other Approaches

Three other models for counseling that can be very useful in groups are solution-focused therapy, the transtheoretical model, and motivational interviewing (MI). All are excellent models regarding how people change.

Solution-Focused Therapy

Solution-focused therapy is an approach to counseling that emphasizes the client's strengths and focuses on solutions (O'Connell, 2005). Solution-focused brief therapy is a very good model for school groups (Murphy, 2005) and many growth, counseling, and therapy groups because of its emphasis on solutions and positive coping. Numerous books and articles have been written about using solution-oriented principles with certain populations (with adolescents—Sklare, 2005; with parents—Zimmerman, Jacobson, Macintyre, & Watson, 1996; substance abuse— Berg & Reuss, 1998; Pichot, 2001; assaultive behavior—Linton, Bischof, & McDonnell, 2005; anger management—Schorr, 1997).

Principles and Techniques Ratner and George (2012) wrote a detailed book about solution-focused principles with one chapter applied to groups. They outline some principles that group leaders can use in their sessions:

- Focus on keeping the group discussions nonpathological.
- Change problems in a way that opens possibilities.
- Focus on exceptions to the problem.
- Comment on members' strength and coping.
- Avoid the tendency to focus on insight into the problem.
- Focus on positive coping behavior.
- View members as people with complaints about their lives and not as people with overwhelming problems.
- Assist members in finding "simpler" solutions.
- Encourage group members to ease into solutions gradually.

Solution-focused group leaders pay much attention to the climate of the group and make sure that the discussion follows these principles. Group leaders using these solution-focused principles concentrate the members on collaboration and solutions; that is, members try to help the working member see the possible solutions and opportunities instead of focusing on insight or the past.

There are also a number of excellent questions that group leaders use when working with an individual or the entire group. These questions get members to understand their goals and their strengths and help members focus on the possibility of and desire to change.

1. **Miracle question**. The miracle question is used to establish goals of a working member or for all the members. De Shazer (1991) describes the miracle question this way: Suppose that one night there is a miracle and while you were sleeping the problem that brought you to therapy is solved. How would you know? What would be different? (p. 113).

By using this question, the leader can get the member or members to clarify what it is that they want to be different and what that would look like.

Group leaders using solution-focused techniques will frequently pose the miracle question.

2. **Exception questions**. De Shazer and Dolan (2007) discusses the value of getting members to think about times when the problem is not a problem or when it is less of a problem. Linton et al. (2005) list a number of excellent exception questions for group leaders to use in a group for assaultive behavior:

- What is different about the times when you manage your anger effectively?
- What is happening during those times when you would expect you might lose your temper, but you do not?
- How often does that happen?
- What has to happen for that to occur more often?
- What would be a small step, something you could do this next week, that would help that happen?

Exception questions get members to think differently about their problems; the questions focus on making changes that will help members reach their desired goals.

3. **Scaling questions**. De Shazer and Dolan (2007) write about the value of having members use numbers instead of words, especially when members are having trouble expressing themselves in words. Linton et al. (2006) offer some sample questions that get members thinking about change:

- On a 1–10 scale, with 10 being the most desirous, how would you rate your desire to change?
- On a 1–10 scale, with 10 being the most confident, how confident are you that you will be successful for 2 weeks?
- On a 1–10 scale, with 10 being the most confident, how confident would your friends be that you will be successful?

These kinds of scaling questions help members to assess where they are in regards to changing and they tend to give members clarity.

4. **Coping questions**. Linton et al. (2006) state that coping questions are helpful when attempts to elicit exceptions and future possibilities are met with negativity and a denial that anything positive is happening or could ever happen (p. 16). Coping questions would be something like:

- What keeps you going?
- What are you doing to keep things from getting worse?
- How do you get yourself to these group sessions?

The goal of coping questions is to heighten members' awareness of the effort that it takes to keep things the same or to cope with their problems. Coping questions also give recognition to the person's effort and highlight the fact that he is doing *something*.

Each of these questions is excellent and helps members focus on solving their problems rather than dwelling on them. Too, by using solution-focused principles as the primary approach in the group, the leader gets members to help other members both see possible solutions and emphasize their strengths instead of their weaknesses.

For those interested in using solution-focused techniques, we strongly encourage you to read articles and books on solution-focused therapy that pertain to the population in which you are interested.

Transtheoretical Model

In their theories book *Systems of Psychotherapy: A Transtheoretical Analysis*, Prochaska and Norcross (2014) present the *transtheoretical model*, which utilizes all of the theories and accounts for how people change. In the transtheoretical model, Prochaska and Norcross propose that people go through stages of change, and knowing these stages of change helps any counselor in both individual and group counseling. Much has been written about stages of change and we believe that *every* counselor and group leader needs to be familiar with these stages. Group leaders can use the information to understand group dynamics that arise because of members' stages of change. It is important for group counselors because members are at various stages of change and if leaders understand this, they will be more efficient and effective.

Stages of Change According to Prochaska and Norcross, change takes place over a series of five stages: precontemplation, contemplation, preparation, action, and maintenance.

Precontemplation is the stage when the person has no interest in or desire to change. The member often does not see that there is a problem. Often members in a nonvolunteer group are in the precontemplation stage in that they do not see themselves as having a problem. Certainly in many addiction groups, the leader will be dealing with members in denial, which is the same as precontemplation.

Contemplation is the stage where members are aware of their problems and are seriously considering changing but have not yet made a commitment to change. It is important to understand that many stay in contemplation for a long time. Groups often help those in contemplation to move to preparation and action because the contemplator sees others making progress and changes—this often is quite motivating and is one of the benefits of being in groups. There will be times when a contemplator will fool a leader and the leader will spend far too much time with the member and thus the group will not be as productive as it could be. By understanding stages of change, the leader will make this mistake less frequently.

Preparation is the stage that combines intention and behavior. The member shows that he wants to change and in fact is taking some steps toward change, such as cutting down on drinking or smoking, or studying at least some each day. Other examples of the preparation stage are investigating different workout gyms or reading up on different diets.

Action is the stage in which members make changes. Often the leader will ask for "progress reports" and those in the action stage tell the different things that they are doing. Members who are in contemplation or preparation often are encouraged by hearing members who are in the action stage. It is important for leaders to remember that the goal of most counseling and therapy groups is to get members into the action stage.

Maintenance occurs in groups if they last long enough. This is the stage where people maintain their change, such as not drinking or not smoking or staying away from pornography. Members who have maintained their changes are an inspiration for those who are in contemplation, preparation, or action.

Use of Stages of Change Leaders can use the transtheoretical stages of change model in many different ways in their groups. We recommend for any growth, counseling, or therapy group that the leader at the very least consider frequently the stage of change of each member as the group progresses or as a topic is introduced. By thinking about the stage of change of the members, the leader may not focus on a certain member because the member seems as if she is in precontemplation or stuck in contemplation. Often in groups, much time is mistakenly spent with members who do not want to change. This is unfortunate, especially if there are members who are in the preparation or action stages of change. Leaders who are always thinking about the stages of change of their group members will not be as inclined to make this mistake. They will focus on members who want to change.

Another way to use the stages of change in groups is to teach the stages to the members so that members can use this model when trying to help themselves or other members. Perhaps the best way to teach this is to have all members think of something they are interested in changing or a problem in their life and also something that they have changed, such as a bad relationship, losing weight, exercising, or quitting smoking. Then teach the stages and discuss which stage members are in regarding their particular situations. Also, get the members to reflect on the change process they went through in reference to the thing that they changed. By spending time discussing the stages, members come to understand them and the leader can use the stages in later sessions as members bring up issues to work on. *A skilled group leader is always thinking about the stage of change of any member who is working on some issue in the group.*

Motivational Interviewing

We want to briefly mention an approach that utilizes the principles of stages of change that is used in many kinds of addiction-oriented groups and is expanding to other kinds of groups. MI is a counseling approach developed by psychologists William R. Miller, Ph.D., and Stephen Rollnick (Miller & Rollnick, 2012).

The primary objective of MI is to increase the internal behavior. MI is very much a goal-oriented approach change by helping group members explore and resolve a much less on the past and much more on the present.

The success of MI requires maintaining a positive and collab... in the group. The leader is watchful that a "negative" member doesn't projec... his or her resistance onto the group. A negative member may try to sabotage the group by making statements such as, "I've been through this group stuff before and none of this works," or "These guys talk a good line but I know they just say what will get them out of here." Although it can be true that some group members will "play the system," the effective group leader using MI deals with this resistance by maintaining her collaborative, objective manner and asks the kinds of questions that point out inconsistencies.

We believe that MI can be a very effective approach in certain group situations. For those interested in learning more, we suggest you start with Miller and Rollnick's book, *Motivational Interviewing* (2012).

Concluding Comments

We cannot stress strongly enough the importance of understanding and being able to apply at least two or three theoretical models when leading counseling and therapy groups. The purpose of this chapter was to give you an overview of several counseling theories that can be used in group counseling situations. These theories were chosen because they are practical, effective, and, for the most part, easy to teach or share with group members. There are numerous others that could have been included but our purpose here was mainly to introduce some theories that we feel are particularly useful for many different kinds of groups.

TA and REBT are psychoeducational models that members can learn relatively easily and use with each other in the group and in their daily lives. Both models are effective with children and adolescents, and offer considerable published materials that can be useful in working with those populations as well as with adults. Reality therapy is also a psychoeducational model that focuses on the here and now, with much responsibility placed on the member to determine her needs, and to develop a plan to meet those needs. The Gestalt model provides strategies for deepening the focus in counseling and therapy groups. Several Gestalt techniques, such as the empty chair, can be combined with other theories. The Adlerian model is excellent for helping members gain a broad perspective on how family dynamics may have influenced their current behavior. This model can be used with REBT as members work toward disputing the irrational beliefs that support mistaken goals and guiding fictions. Solution-focused therapy is an excellent approach that focuses on possibilities and solutions. The transtheoretical model and motivational interviewing emphasize the stages of change, which can be helpful to both the leader and the members.

ACTIVITIES

1. Imagine three critical moments in a group that you might be leading and then think of how you might use one or more of the theories described in this chapter to work through the various issues connected to the critical moments.
2. Discuss with fellow students or colleagues their use of theories in groups. Especially look for those who have used counseling theories in group settings.
3. Discuss the advantages and disadvantages of using just one theory in group sessions.



GROUP COUNSELING SKILLS

View again the three videos using theories 13.1, 13.2, and 13.3 and think about other ways to use these theories. Think of how these groups may have gone if theory was not utilized.